Registry pilot · Mission 2 · A paper for your board

What a registry pilot involves for your association

DysNet is building the first international registry of limb malformations owned by the people it describes. The Annual General Meeting of 26 August 2026 created it and mandated Health Data Safe, a Swiss non-profit foundation, as its technical and operational partner. Assedea in France and Raggiungere in Italy launch it. This paper answers, without a meeting, what taking part would ask of your association.

Why a registry, and why now

Between 2007 and 2014 three clusters of transverse upper-limb agenesis came to light in France, in Loire-Atlantique, Ain and Morbihan. The investigations found no common exposure, and they met the limit of any retrospective enquiry: families were questioned years after the birth, from memory. Families who record circumstances and exposures as they happen, from pregnancy onwards, give the next investigation what the last one lacked. No registry at national or European level is dedicated to limb anomalies.

What your association gives

A point of contact

One person who answers our mail and carries the pilot inside your association. Not a project team.

Your language

Help turning the questions into the words your families actually use, so an Italian record and a French one mean the same thing.

An invitation, not a list

You invite your families. Nobody is entered by their association, and we never ask you for your membership file.

Patience with a first version

A pilot exists to be corrected. We would rather you told us the questions are wrong than watched you answer them anyway.

What your association gets

The figures for your own country

Most associations have never had them. You would be able to say how many, where, and with what care, and say it with a source.

A seat on the research question

Which studies may be put to your families at all is decided by the member associations and the DysNet board together, not by whoever asks first.

Data your families own

Each person holds their own account and decides what enters the registry. Your relationship with them is not mediated by us.

An infrastructure you do not run

No servers, no hosting contract, no data protection officer to recruit. That burden sits with the technical partner.

The questions your board will ask

Who owns the data?

The person described by it. Not DysNet, not your association, and not the technical partner. Each person holds their own account, decides what enters the registry, and can withdraw.

What does it cost us?

No licence fee and no charge per family. Health Data Safe contributes its infrastructure in kind rather than as a commercial service. The real cost is volunteer time: a contact, the translation, and the invitations.

Do we hand over our member list?

No. We never ask for it. Families are invited by you and enter their own information themselves, which is also why the registry describes a community rather than counting a population.

What if a family changes its mind?

Consent is given study by study and can be withdrawn at any time, in the family's own language, under the GDPR rules your association already works with.

What if Health Data Safe disappears?

Its statutes provide that personal data is never treated as an asset of the foundation, and that on dissolution the data is destroyed or moved to a service offering similar guarantees. The data model is built for portability for the same reason.

Does this compete with our national registry?

No. Population registries count a catchment area from the clinical side. This records what families declare, from pregnancy onwards, including the people who never reach a catchment area at all.

How a pilot runs

1 **One call**
We show you the questions, and what a single record actually looks like. Nothing is signed.

2 **The questions in your language**
You correct the wording until it matches how your families speak about themselves.

3 **A handful of families, not a launch**
You invite a small group. A pilot is meant to find what is wrong while it is still cheap to change.

4 **Each family opens its own account**
They decide what goes in, and they consent, or they do not. You are not asked to vouch for anyone.

5 **Your country's figures come back to you**
The first thing a pilot produces is a picture of your own membership that you did not have before.

6 **Then it becomes findable**
The registry has been declared to Orphanet's European directory of rare-disease registries, where researchers already look.

Where it stands today

26 August 2026. The Annual General Meeting adopted the refocused strategy, created the registry and mandated Health Data Safe as its technical and operational partner.

September 2026. European rare-disease calls are being screened, and the registry is seeking its first funders. Assedea and Raggiungere carry the two launch pilots.

What we are careful not to claim

Measured against the definition of a patient registry adopted by the European recommendations on rare-disease registries, what we are building is not yet a registry, and we would rather say so than borrow the word's authority.

• It is not exhaustive. A registry aims at every case in a defined population; ours gathers the families who choose to take part.

• There is no medical board yet. Writing a protocol and choosing what is worth collecting are scientific acts, and an association without a scientific committee will not pretend to perform them.

• Declared data is not clinician-verified data. What families report about themselves is precious, and the two must never be confused.

Earning the word is the roadmap: a scientific committee, a written protocol, an agreed minimum data set and a published quality plan.

What we are asking of you

Tell us whether your board wants a conversation. A pilot starts with one call, a look at the questions in your language, and a date. Write to info@dysnet.org with the word **pilot** in the subject line.

While you decide, everything else we maintain is already free to use: a bibliography of 1,934 references, a teratogens register of 587 substances with the decisions authorities have taken on them, the care centres, the researcher register and the epidemiology tables, at www.dysnet.org/knowledge/.